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Talab Road,Raipur-492001, Chhattisgarh

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Patient Name	:	DR ALOK KUMAR JAIN	**Registered On**	:	15/10/2025 04:27PM
Patient Id	:	401025115400	**Sample Collected On**	:	15/10/2025 04:16PM
Age/DOB/Gender	:	62Y /0M /0Days/ {MALE}	**Reported On**	:	15/10/2025 05:41PM
Nationality	:	INDIAN	**Sample UID No.**	:	P011B000428314
Customer Type	:	B2B - Lab	**Customer Name**	:	DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	:	SELF			

Glycosylated Haemoglobin (HbA1c)

Investigation	Result	Units	Biological Reference Interval
Glycosylated Haemoglobin(HBA1C)	**7.4**	%	Non-diabetic adults>=18 years <5.7 At risk (Pre-diabetes) 5.7-6.4 Diagnosing Diabetes >=6.5 Therapeutic goals for glycemic control Age> 19 years:< 7 For Age< 19 year:< 7.5 .
Mean Blood Glucose	165.68	mg/dl	

Sample Type : EDTA Whole Blood.
Method : High-performance liquid chromatogrphy.

Interpretations:

Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled . Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate. Comments: HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations Disclaimer: The above result relate only to the specimens received and tested in laboratory and should be always correlate with clinical findings and other laboratory markers. Improper specimen collection, handling, storage and transportation may result in false negative/Positive results.

Disclaimer :

1) The above result relate only to the specimens received and tested in laboratory and should be always correlate with clinical findings and other laboratory markers.
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Patient Name	: DR ALOK KUMAR JAIN	**Registered On**	: 15/10/2025 05:43PM
Patient Id	: 401025115400	**Sample Collected On**	: 15/10/2025 04:16PM
Age/DOB/Gender	: 62Y /0M /0Days/ {MALE}	**Reported On**	: 15/10/2025 06:40PM
Nationality	: INDIAN	**Sample UID No.**	: P011B000428314
Customer Type	: B2B - Lab	**Customer Name**	: DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	: SELF		

Microalbumin / Creatinine Ratio

Investigation	Result	Units	Biological Reference Interval
Microalbumin • Method : Turbidimetric Immunoassay.	**2.1**	mg/dL	< 1.7
Creatinine Urine • Method : Jaffe Method.	137.61	mg/dl	15 - 278
Microalbumin / Creatinine Ratio • Method : Calculated.	15.26	mg/g	

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Patient Name	:	DR ALOK KUMAR JAIN	**Registered On**	:	15/10/2025 04:27PM
Patient Id	:	401025115400	**Sample Collected On**	:	15/10/2025 04:16PM
Age/DOB/Gender	:	62Y /0M /0Days/ {MALE}	**Reported On**	:	15/10/2025 05:03PM
Nationality	:	INDIAN	**Sample UID No.**	:	P011B000428314
Customer Type	:	B2B - Lab	**Customer Name**	:	DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	:	SELF			

Electrolytes

Investigation	Result	Units	Biological Reference Interval
Sodium • Sample Type : Serum.	144.4	mmol/L	135- 145
Potassium • Sample Type : Serum. • Method: Indirect ISE.	**3.32**	mmol/L	3.5 - 5.5
Chloride • Sample Type : Serum. • Method : ISE Direct.	106.2	mmol/L	

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Patient Name	:	DR ALOK KUMAR JAIN	**Registered On**	:	15/10/2025 04:27PM
Patient Id	:	401025115400	**Sample Collected On**	:	15/10/2025 04:16PM
Age/DOB/Gender	:	62Y /0M /0Days/ {MALE}	**Reported On**	:	15/10/2025 05:41PM
Nationality	:	INDIAN	**Sample UID No.**	:	P011B000428314
Customer Type	:	B2B - Lab	**Customer Name**	:	DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	:	SELF			

CK-MB (MB fraction of Creatinine Kinase)

Investigation	Result	Units	Biological Reference Interval
CK-MB (MB fraction of Creatinine Kinase)	1.35	ug/l	0 - 25

• Sample Type : Serum.
• Method : ECLIA.

• Disclaimer :
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2) Improper specimen collection,handling,storage and transportation may result in false negative/Positive results.

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Patient Name	: DR ALOK KUMAR JAIN	**Registered On**	: 15/10/2025 04:27PM
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Nationality	: INDIAN	**Sample UID No.**	: P011B000428314
Customer Type	: B2B - Lab	**Customer Name**	: DR. BR AMBEDKAR CLINICAL LAB RAIPUR
Ref. Doctor Name	: SELF		

PSA - Total Prostate Specific Antigen

Investigation	Result	Units	Biological Reference Interval
PSA - Total Prostate Specific Antigen	0.41	ng/mL	<4.1

• Sample Type : Serum.

• Method : ECLIA.

• Comments :
This is a recommended test for detection of prostate cancer along with Digital Rectal Examination (DRE) in males above 50 years of age. PSA levels may appear consistently elevated / depressed due to the interference by heterophilic antibodies & nonspecific protein binding. Immediate PSA testing following digital rectal examination, ejaculation, prostatic massage, indwelling catheterization, ultrasonography and needle biopsy of prostate is not recommended as they falsely elevate levels. PSA values regardless of levels should not be interpreted as absolute evidence of the presence or absence of disease. All values should be correlated with clinical findings and results of other investigations. Sites of Non-prostatic PSA production are breast epithelium, salivary glands, periurethral & anal glands, cells of male urethra & breast milk. Physiological decrease in PSA level by 18% has been observed in hospitalized / sedentary patients either due to supine position or suspended sexual activity.

• Clinical Use :
1)An aid in the early detection of Prostate cancer when used in conjunction with Digital rectal examination in males more than 50 years of age and in those with two or more affected first degree relatives.
2)Followup and management of Prostate cancer patients.
3)Detect metastatic or persistent disease in patients following surgical or medical treatment of Prostate cancer Increased Levels seen in Prostate cancer, Benign Prostatic Hyperplasia, Prostatitis, Genitourinary infections.

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